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Product : T - CHLORAMINE SYRUP [FRONT] SAP No. : Date : 03 April 2026		☐ Unit Box ☐ Outer Box ☐ Label ☒ Leaflet ☐ Fix-A-Form ☐ Shipper Carton ☐ Aluminium Foil ☐ Sachet ☐ Others : _____			
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CHLORAMINE SYRUP

DESCRIPTION

Chloramine Syrup 4mg/5ml

A yellow, orange flavoured syrup.

Each 5 ml contains Chlorpheniramine maleate 4 mg.

Preservative: Methyl Paraben 0.21%w/v Propyl Paraben 0.031%w/v

Chloramine Syrup 2.5mg/5ml

A reddish orange, orange flavoured syrup.

Each 5 ml contains Chlorpheniramine maleate 2.5 mg.

Preservative: Methyl Paraben 0.21% w/v & Propyl Paraben 0.031%w/v

INDICATIONS

Chlorpheniramine is used for the prophylactic and symptomatic treatment of perennial and seasonal allergic rhinitis, vasomotor rhinitis, allergic conjunctivitis due to inhalant allergens and foods and sneezing and rhinorrhoea associated with the common cold. It is also indicated for the symptomatic relief of pruritus associated with allergic reactions and of mild, uncomplicated allergic skin manifestations of urticaria and angioedema, in dermatographism, and in urticaria associated with transfusions. It is used as adjunctive therapy to epinephrine and other standard measures for anaphylactic reactions after the acute manifestations have been controlled, and to ameliorate the allergic reactions to blood or plasma.

PHARMACODYNAMICS

Chlorpheniramine is a propylamine derived antihistamine. Its use in the treatment of allergy acts by competing with histamine H1-receptor sites on effector cells. It does not inactivate histamine or prevent its synthesis or release. It thereby prevents, but do not reverse, responses mediated by histamine alone. Antihistamines antagonise, in varying degrees, most of the pharmacological effects of histamine, including urticaria, pruritus and vasodilation.

PHARMACOKINETICS

Chlorpheniramine maleate is absorbed relatively slowly from the gastro-intestinal tract, peak plasma concentrations occurring about 2-6 hours after administration by mouth while peak effect occurs at 6 hours after administration. The onset of action of chlorpheniramine is between 15-60 minutes. Bioavailability is low, values of 25%-50% having been reported. About 72% of chlorpheniramine in the circulation is bound to plasma proteins. There is wide interindividual variation in the pharmacokinetics of chlorpheniramine; values ranging from 14-25 hours have been reported for the half-life. Chlorpheniramine is widely distributed in the body, including passage into the CNS. Chlorpheniramine maleate is extensively metabolised. Metabolites include desmethyl- and didesmethylchlorpheniramine. Unchanged drug and metabolites are excreted primarily in the urine; excretion is dependent on urinary pH and flow rate. Only trace amounts have been found in the faeces. A duration of action of 4-6 hours has been reported; this is shorter than may be predicted from pharmacokinetic parameters. More rapid and extensive absorption, faster clearance and a shorter half-life have been reported in children.

DOSAGE

Adult

Usual dose: 4 mg every 4-6 hours as needed.

Usual adult prescribing limit: Up to 24 mg daily.

Pediatric

Children 6-12 years: 2 mg every 4-6 hours as needed, not to exceed 12 mg daily.

Children 2-5 years: 1 mg every 4-6 hours as needed, not to exceed 6 mg daily.

Geriatric

Usual adult dose.

Geriatric patients may be more sensitive to the effects of the usual adult dose.

Note: For children under 2 years, please refer to Warning and Precautions.

SIDE EFFECTS

Drowsiness and thickening of mucous occurs frequently with chlorpheniramine. Blurred vision, confusion, difficult or painful urination, dizziness, dry mouth, nose, or throat, tachycardia, increased sweating, loss of appetite, paradoxical reactions (nightmares, unusual excitement, nervousness, restlessness or irritability), photosensitivity (increased sensitivity of skin to sun), ringing or buzzing in ears, skin rash, stomach upset (nausea, vomiting, diarrhoea, epigastric pain) and constipation have also been reported. These effects are more likely to occur in children and elderly patients. Blood dyscrasias including agranulocytosis, leucopenia, haemolytic anaemia, and thrombocytopenia (sore throat and fever, unusual bleeding or bruising, unusual tiredness or weakness) though rare, have been reported. Other rare side effects include hypotension, headache and paraesthesia.

CONTRAINDICATIONS

Chlorpheniramine should not be given to premature infants or neonates; this group of patients has an increased susceptibility to antimuscarinic effects. Those that have MAOI's therapy within the previous 14 days.

PRECAUTIONS

The anticholinergic properties of chlorpheniramine may cause drowsiness, dizziness, blurred vision and psychomotor impairment in some patients which may seriously affect ability to drive or operate machinery. Anticholinergic effects may also precipitate or aggravate urinary retention in patients with bladder neck obstruction, symptomatic prostatic hypertrophy and predisposition to urinary retention. Increased intraocular pressure may precipitate an attack of angle-closure glaucoma. Chlorpheniramine should be used with care in patients with pyloroduodenal obstruction, severe cardiovascular disorders and epilepsy. Patients sensitive to one antihistamine may be sensitive to chlorpheniramine as well. Antihistamines may inhibit the cutaneous histamine response, thus producing false-negative results; it is recommended that chlorpheniramine be discontinued at least 72 hours before testing begins.

When used for treatment of cough and cold:

- Not to be used in children less than 2 years of age
- To be used with caution and doctor/pharmacist's advice in children 2 to 6 years of age.

DRUG INTERACTIONS

Concurrent use with alcohol and CNS depressants such as barbiturates, hypnotics, opioid analgesics, monoamine oxidase (MAO) inhibitors, anxiolytic sedatives and neuroleptics may potentiate the CNS depressant effects. Anticholinergic effects may be potentiated when these medications such as atropine, tricyclic antidepressants and MAO inhibitors are used concurrently with antihistamines. Concurrent use with ototoxic medications such as aminoglycoside antibiotics with antihistamines may mask the symptoms of ototoxicity such as tinnitus, dizziness and vertigo. Additive photosensitising effects may occur with use of these medications with antihistamines. Chlorpheniramine inhibits phenytoin metabolism and can lead to phenytoin toxicity.

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OVERDOSAGE

Clumsiness or unsteadiness, severe drowsiness, severe dryness of mouth, nose or throat, flushing or redness of face, shortness of breath or troubled breathing are signs and symptoms of anticholinergic effects. Cardiac arrhythmia (fast or irregular heartbeat), CNS stimulation (hallucinations, seizures, trouble in sleeping, ataxia, excitement, tremors, psychoses) and hypotension are all signs and symptoms of overdosage. Anticholinergic and CNS stimulant effects are more likely to occur in children with overdose while hypotension may also occur in the elderly at usual doses. Overdose may be fatal especially in infants and children.

Since there is no specific antidote for overdose with chlorpheniramine, treatment is symptomatic and supportive with possible utilisation of the following:

- a) Induction of emesis (syrup of ipecac recommended); however, precaution against aspiration is necessary, especially in infants and children.
- b) Gastric lavage (isotonic or 0.45% sodium chloride solution) if patient is unable to vomit within 3 hours of ingestion.
- c) Saline cathartics (milk of magnesia) and activated charcoal are sometimes used, especially if sustained-release preparations were involved.
- d) Vasopressors such as norepinephrine or phenylephrine to treat hypotension; however, epinephrine should not be used since it can paradoxically lower the blood pressure further.
- e) Oxygen, external cooling for hyperpyrexia and intravenous fluid.
- f) Precaution against use of stimulants (analeptic agents) because they may cause seizures.

Up-to-date information on treatment of overdose can be obtained from The National Poison Centre, University Sains Malaysia.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Chloramine may cause drowsiness, dizziness, blurred vision and psychomotor impairment in some patients which may seriously affect ability to drive or operate machinery

TOXICOLOGY

Cross sensitivity and/ or related problems: Patients sensitive to one of the antihistamines may be sensitive to others.

Carcinogenicity/ Tumorigenicity/ Mutagenicity: Long term animal studies to evaluate carcinogenic, tumorigenic, or mutagenic potential of most antihistamines have not been performed.

Pregnancy/Reproduction: Studies with chlorpheniramine in humans have not been done. Studies in animals have not shown that chlorpheniramine causes adverse effects on the foetus. (FDA Pregnancy Category B)

Breast-feeding: First generation antihistamines may inhibit lactation because of their anticholinergic actions. Small amounts of antihistamines are distributed into breast milk; use is not recommended in nursing mothers because of the risk of adverse effects, such as unusual excitement or irritability, in infants.

Pediatrics: Use is not recommended in newborn or premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation, and an increased tendency toward convulsions. A paradoxical reaction characterized by hyperexcitability may occur in children taking antihistamines.

Geriatrics: Dizziness, sedation, confusion, and hypotension may be more likely to occur in geriatric patients taking antihistamines. A paradoxical reaction characterized by hyperexcitability may occur in geriatric patients taking antihistamines. Geriatric patients are especially susceptible to the anticholinergic side effects, such as dryness of mouth and urinary retention (especially in males) of the antihistamines. If these side effects occur and continue or severe, medication should probably be discontinued.

Dental: Prolonged use of antihistamines may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis, and discomfort.

PRESENTATION

Chloramine Syrup 4mg/5ml } 90ml
Chloramine Syrup 2.5mg/5ml } 90ml

STORAGE CONDITIONS AND USER INSTRUCTIONS

Keep container tightly closed.
Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jaubi daripada kanak-kanak.
Route of administration: Oral
Shelf life: Please refer to outer package.
May be taken with food, water or milk to lessen gastric irritation.
Do not take more medicine than the amount recommended.
May cause drowsiness. If affected, do not drive or operate machinery.
Avoid alcoholic beverages.

Product Registration Holder:
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Section 13, Bangi Industrial Estate,
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Revision Date: April 2026

